

2026 Western Dental Evidence of Coverage



CALIFORNIA DEPARTMENT OF HUMAN RESOURCES

**COMBINED EVIDENCE OF COVERAGE AND
DISCLOSURE FORM
DENTAL CARE PLAN ISSUED BY
WESTERN DENTAL SERVICES, INC.
P.O. BOX 14227
ORANGE, CA 92863
(866) 859-7525**

Group Plan

This combined evidence of coverage and disclosure form constitutes only a summary of the dental plan. The dental plan group subscriber agreement must be consulted to determine the exact terms and conditions of coverage. A specimen copy of the dental group subscriber agreement will be furnished upon request.



WELCOME TO WESTERN DENTAL.

This Evidence of Coverage Booklet, which includes the Combined Evidence of Coverage and Disclosure Form and the accompanying Schedule of Benefits, describes the dental plan being offered by Western Dental Services, Inc., and discloses the terms and conditions of coverage. All applicants have the right to review this Evidence of Coverage Booklet prior to enrollment. Western Dental is called the “Plan” throughout this Evidence of Coverage Booklet.

The Evidence of Coverage Booklet explains your rights and responsibilities as a Western Dental Member. It also explains the Plan’s responsibilities to you. The Evidence of Coverage Booklet contains important information and should be read completely and carefully. Individuals with special health needs should read carefully those sections that apply to them. Please keep the Evidence of Coverage Booklet in a safe place, available for quick reference. If you would like to receive additional information about the benefits of enrollment in Western Dental, please call us at the number above.

This Evidence of Coverage Booklet does not take effect until the Group Subscriber Agreement (“Agreement”) between the State of California (“Group”) and Western Dental is approved and executed by the Plan and the Group. This Benefit Plan shall be construed under the laws of the State of California; and any action relating to this Benefit Plan shall be instituted and prosecuted in the county in which the Member resides at the time the Agreement is executed or in such other location as the parties may mutually agree in writing.

Please Note: Except for Emergency Dental Care and services prior authorized by the Plan to be provided by non-Participating Providers, the Covered Services under this Benefit Plan are available only when provided by Participating Providers in accordance with all the terms and conditions of coverage described in this Evidence of Coverage Booklet and the Agreement.

It is your responsibility to determine whether the dentist or specialist dentist you use is a Participating Provider. It is also your responsibility to determine whether or not a referral made by your dentist or Participating Provider is to a Participating Provider. Even though your dentist may be a Participating Provider, do not assume that his or her referral to another dentist/specialist or facility is a determination that such dentist/specialist or facility is also a Participating Provider. If

you are in doubt about the status of any dentist or facility call the Plan's Member Service Department for verification.

Multilingual Services

If you or your representative prefer to speak in any language other than English, please call Western Dental, toll-free, at (866) 859-7525 to speak with a Western Dental Member Service Representative. Our Member Service staff can help you find a Participating Provider who speaks your language or who has a regular interpreter available. You do not have to use family members or friends as interpreters. If you cannot locate a Participating Provider who meets your language needs, you can request to have an interpreter available for discussions of medical information at no charge.

This EOC booklet, as well as other information materials, has been translated into Spanish. To request translated materials, please call Western Dental, toll-free, at (866) 859-7525.

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DEFINITIONS

Adult Dentition means the teeth that are present after the cessation of growth that would affect Orthodontic treatment.

Aesthetic Dentistry means any dental procedures, which are performed purely for cosmetic purposes, and where there is no restorative value.

Agreement means the Group Subscriber Agreement between the State of California (Group) and the Plan.

Benefit Plan means the specialized dental plan offered by Western Dental Services, Inc. pursuant to the requirements of the Knox-Keene Health Care Service Plan Act and regulations promulgated there under.

COBRA refers to the Consolidated Omnibus Budget Reconciliation Act of 1986, enacted April 7, 1986.

Copayment means the fee charged to the Member by the Participating Provider, as described in this Evidence of Coverage Booklet, the Agreement and the Schedule of Benefits.

Covered Services means the dental services available under the Benefit Plan in which a Member is enrolled.

Dependent means the spouse or Domestic Partner of a Subscriber and the natural, adopted, step or Domestic Partner's children, as defined herein under the section entitled Eligibility.

Domestic Partner means an adult who is in a domestic partnership with the Subscriber, which domestic partnership meets the criteria set forth in California Family Code Section 297, et. seq.

Elective Dentistry means any dental procedures, which are unnecessary to the dental health of the Member, as determined by a Participating Provider.

Eligible Participants means employees or beneficiaries of Group, and their Dependents, who are eligible to participate in the Benefit Plan under the eligibility requirements, set forth by Group.

Emergency Dental Care means services to diagnose and treat a dental condition, which is manifested by acute symptoms of sufficient severity, including severe pain, such that a prudent lay person with no special knowledge of dentistry could reasonably expect the absence of immediate dental attention to result in:

- Placing the health of the individual (or, in the case of a pregnant woman, the health of the woman or her unborn child) in serious jeopardy
- Serious impairment of bodily functions
- Serious dysfunction of any bodily organ or part.

Exclusion means any provision of the Benefit Plan whereby coverage for a specified hazard or condition is entirely eliminated.

General Practitioner means a dentist who practices general dentistry and who does not hold himself out to be a specialist in a particular field of dentistry.

Group means the State of California.

Limitation means any provision, other than an Exclusion, which restricts coverage under the Benefit Plan.

Member means an Eligible Participant who is enrolled in the Benefit Plan, and for whom Prepayment Fees have been paid to the Plan by the Group.

Participating Provider means the Benefit Plan through its employed dentists, or a dentist under contract with the Plan as a General Practitioner and/or a Specialist.

Plan means Western Dental Services, Inc. ("Western Dental").

Prepayment Fee means the amount payable each month on a prepayment basis by a Member or the Group (or both) to obtain benefits provided under the Benefit Plan.

Primary Dentition means teeth developed and erupted first in order of time.

Schedule of Benefits means the list of Covered Services, and the authorized Copayment amounts under the Benefit Plan as set forth in this EOC.

Specialist means a dentist who is a Participating Provider who is responsible for the Specific Specialized Dental Care of a

Member in one specific field of dentistry such as endodontics, periodontics, oral surgery, or orthodontics where the Member is referred by a Participating Provider.

Specific Specialized Dental Care means the Covered Services diagnosed and administered to a particular Member by a Specialist, which a Member receives as a result of a referral to the Specialist or other Participating Provider.

Subscriber means the individual enrolled in the Benefit Plan for whom the appropriate Prepayment Fee has been received by the Plan, and whose employment or other status, except for family dependency, is the basis for enrollment eligibility.

Transitional Dentition means the final phase of the transition from primary to adult teeth, in which the deciduous teeth are in the process of shedding and the permanent successors are emerging.

ACCESS TO SERVICES

Facilities

Members may obtain a list of the Plan's Participating Providers by calling the Member Service Department or by visiting the Plan's website at www.westerndentalbenefits.com. Participating Providers are open during normal business hours as specified in the Participating Provider listing. Should a Member have a question regarding the days and/or hours of the Participating Provider's facility, he/she may reference the Provider listing or may write or call either the Participating Provider at the address and telephone number specified on the Provider list or the Plan at the address and telephone number listed in this Evidence of Coverage Booklet.

A Member may receive Emergency Dental Services after regularly scheduled office hours by calling the local telephone number for the Participating Provider's facility. The Member will be charged the applicable Copayment as specified in the Schedule of Benefits for "Office Visit - After Regular Scheduled Hours (ADA procedure code D9440)."

Choice of Provider

PLEASE READ THE FOLLOWING INFORMATION SO YOU WILL KNOW FROM WHOM OR WHAT GROUP OF PROVIDERS YOU MAY RECEIVE BENEFITS AND COVERAGE.

Each Member must receive Covered Services from a Participating Provider. A Member may choose any Participating Provider who is available. The Member should review the Plan's most current *Provider Directory* for the Plan that covers the Member to learn who may be available. Once a Member has chosen a Participating Provider, the Member should contact the Participating Provider to receive Covered Services.

Each Member should choose his or her Primary Care Dentist on his or her enrollment form. If the Member does not choose a Primary Care Dentist, the Plan will do so. If a Member wants to change his or her Primary Care Dentist, the Member should contact the Plan. If the request for transfer is received by the Plan by the 20th day of the month, this transfer will become effective on the first day of the following month.

Liability of Member for Payment

By statute, every contract between the Plan and a Participating

Provider shall provide that in the event the Plan fails to pay the Participating Provider, the Member shall not be liable to the Participating Provider for any sums owed by Plan.

In the event the Plan fails to pay non-contracting providers, the Member may be liable to the non-contracting provider for cost of services.

Provider Reimbursement

Services provided by a non-participating provider are not covered under the Benefit Plan. Some Participating Providers are employees of the Plan. The Plan pays each Participating Provider who is an employee a set amount for each day he or she works. The Plan will not pay a bonus to anyone to deny, reduce, limit, or delay the provision of Covered Services that a Member is entitled to receive.

Some Participating Providers are independent dentists under contract with the Plan. The Plan pays those Participating Providers based on the agreements reached with them. The amount the Participating Provider will receive might not depend on the nature or amount of services provided to a Member, as is true with capitation payments.. On the other hand, the amount the Participating Provider will receive (for example, fee- for-service payments) might depend entirely on the nature and amount of services provided.

TIMELY ACCESS TO CARE

Urgent Dental Care

Western Dental Plan covers urgent dental care. If you need to see your Primary Care Dentist right away but it is not an emergency, urgent care appointments are available within 72 hours.

During normal office hours, call your Primary Care Dentist for help. If it is after office hours, try calling your Primary Care Dentist first. If you cannot reach your Primary Care Dentist, call Western Dental Member Services at 1-866-859-7525 (TTY 714-571-3624) anytime for assistance.

Emergency Dental Care

Western Dental Plan covers emergency dental care. A dental emergency can be pain, bleeding, or swelling that can cause harm to you or your teeth if not fixed right away. Emergency dental care is available 24 hours a day, 7 days per week. You do not need approval from Western Dental to get emergency care from your Primary Care Dentist. During normal office hours, call your Primary Care Dentist for help. If it is after office hours, try calling your dentist first. If you cannot reach your Primary Care Dentist, call Western Dental Member Services at 1-866-859-7525 (TTY 714-571-3624) any time for assistance.

Non-urgent Dental Care

If your care is for non-Urgent Dental Care services, your Primary Care Dentist is obligated to see you within ten (10) business days of the request for an appointment. Upon showing that you will not suffer any detrimental impact to your health, your Primary Care Dentist may extend the waiting time for a non-urgent dental care appointment to thirty-six (36) business days of request for an appointment. Unless requested by you to accommodate your schedule, in no case shall a Primary Care Dentist schedule an appointment that is not within the required time periods of the request for an appointment. If a Primary Care Dentist is unable to see you within the time periods noted above, please contact a Western Dental representative immediately at 1-866-859-7525. The Western Dental representative shall ensure that you are treated within the required time periods.

Preventative Dental Care

If your care is for preventative dental care services, your Primary Care Dentist is obligated to see you within ten (10) business days of the request for an appointment. Upon showing that you will not suffer any detrimental impact to your health, your Primary Care Dentist may extend the waiting time for a preventative dental care appointment to forty (40) business days of request for an appointment. Unless requested by you to accommodate your schedule, in no case shall a Primary Care Dentist schedule an appointment that is not within the required time periods of the request for an appointment. If a Primary Care Dentist is unable to see you within the time periods noted above, please contact a Western Dental representative immediately at 1-866-859-7525. The Western Dental representative shall ensure that you are treated within the required time periods.

Interpretive Services

Whether you require routine or Urgent Dental Care and you need any assistance communicating with your Primary Care Dentist, Western Dental shall make available at no cost to you an interpreter. Prior to your appointment, please contact a Western Dental representative at 1-866-859-7525. The Western Dental representative will make all necessary arrangements to ensure that an interpreter is present while you are being treated by your Primary Care Dentist.

Contacting Your Primary Care Dentist

A Western Dental contracted Primary Care Dentist is required by us to have a method in which you can reach him or her during non-business hours in case you are in need of Urgent Dental Care. Your Primary Care Dentist may have an answering machine that he or she checks or an answering service to assist with non-business hour calls. In either case, please leave a detailed message on the answering machine or with the answering service if you require Urgent Dental Care. Your Primary Care Dentist is required to respond to you immediately. If he or she has not responded to your call promptly, please call Western Dental at 1-866-859-7525. A Western Dental representative will respond to your call within 24 hours of the call.

EMERGENCY AND URGENT CARE

Emergency Dental care is available for all Western Dental members 24 hours a day, 7 days a week.

In the event that Member requires urgent or Emergency Dental Care less than 50 miles from his or her Primary Care Dentist during regular Provider office hours, the Member should contact his or her Primary Care Dentist to schedule an immediate appointment. After business hours, the Member should contact his or her Primary Care Dentist, and the on-call dentist will return the call for instructions on how to proceed.

If after contacting his or her Primary Care Dentist the Member is advised that the Primary Care Dentist is not available, the Member may obtain urgent or Emergency Dental Care from any Participating Provider or any licensed dentist in the area where such dental emergency occurs.

Members may contact the Plan for assistance with obtaining an emergency appointment from a Participating Provider. Treatment by Participating Providers will be provided at the applicable Copayment listed in the Schedule of Benefits. Treatment provided by a non-participating provider will be reimbursed up to the maximum of \$100 less the applicable copayments.

However, if a dental emergency occurs while more than 50 miles from the member's Primary Care Dentist, the Member may have emergency services rendered by any licensed dentist in the area where such emergency occurs. The plan will provide a one hundred dollar (\$100) maximum allowable benefit for the following:

1. Control of bleeding
2. Control of infection
3. Relief of pain associated with dental problems

The Plan requires an itemized statement of services from the Non-Participating Provider or the Member within one-hundred eighty (180) days from the date of service for verification of benefit reimbursement.

The Member must include the itemized statement of services, the Member's name, address, Member ID number, dates of service, treating provider's name, address, and telephone number, and a statement of the problem, and mail it to:

Western Dental Services, Inc.
Attn: Specialty Referrals/Claims Department
P.O. Box 14227
Orange, California 92863

The Member should retain a copy of the information, and the Plan will either send the Member a check or explain any denial within thirty (30) business days of the Plan's receipt of the Member's claim.

Members are encouraged to use appropriately the "911" emergency response system, in areas where the system is established and operating, when they have an emergency dental condition that requires an emergency response.

SECOND DENTAL OPINIONS

A Member or a Participating Provider may request a second opinion consultation by writing or calling the Plan's Member Service Department at (866) 859-7525. Decisions and notifications regarding requests for second opinion consultations will be rendered within the following time limits: For routine second opinion requests, the decision to approve or deny requests for second opinion consultations will be made within 5 business days of the Plan's receipt of the request. For urgent requests, the second opinion will be authorized or denied within 72 hours of the Plan's receipt of the request. The requesting Participating Provider will be notified both verbally and in writing within 24 hours of the decision. The decision will be communicated to a requesting Member verbally (when possible) and in writing within 2 business days.

A second opinion consultation may be authorized for surgical procedures, unclear or complex and confusing clinical indications, conflicting test results, the Participating Provider's inability to diagnose the Member's condition, a treatment plan

in progress but not improving the Member's condition within an appropriate time period, or the Member's serious concerns about a particular diagnosis or plan of care. A written Explanation of Benefits will be issued to the Member and the Member's Participating Provider, including the name and location of the second opinion provider if the second opinion is approved. Upon approval, the Plan will refer the Member to a Participating Provider who is under contract with the Plan. Should there be no available Participating Provider in the appropriate geographical area, the Plan will refer the Member to a Non-Participating Provider for a second opinion consultation. A Plan representative will assist the Member in scheduling an appointment or will advise the Member to call and schedule an appointment. The second opinion provider will submit the claim for payment to the Plan. The Member is only responsible for the applicable copayment as set forth in the Schedule of Benefits providing the member obtains referral from Primary Care Dentist. Member may not initiate second opinion without prior approval from either the Participating Provider or the Plan. The Plan will pay any cost in excess of the applicable copayment and will contact the provider rendering the second opinion to advise the provider of Western Dental's payment in excess of the Copayment.

The second opinion provider will provide the Member and the Member's Participating Provider with a written narrative report of the results of the Member's consultation. All treatment must be performed by the Member's Participating Provider for the Member to receive Covered Services under the Benefit Plan. This shall not limit the Member's right to transfer to another Participating Provider in order to receive Covered Services under the Benefit Plan.

CONTINUITY OF CARE

Current Members:

Current Members may be eligible to temporarily continue receiving Covered Services from a non-Participating Provider for treatment of certain specified dental conditions if the services were being provided by a Participating Provider at the time the provider's contract with Western Dental terminated (i.e. a "terminated provider"). Please call the Plan's Member Service Department at (866) 859-7525 to see if you may be eligible for this benefit. You may request a copy of the Plan's Continuity of Care Policy from the Plan's Member Service Department. You must make a specific request to continue under the care of your terminated provider. The Plan is not required to continue your care with your terminated provider if

you are not eligible under the Plan's Continuity of Care Policy or if the Plan cannot reach agreement with your terminated provider on the terms regarding your care in accordance with California law.

New Members:

New Members may be eligible to temporarily continue receiving Covered Services from a non-Participating Provider for treatment of certain specified conditions if the services were being provided by a non-Participating Provider at the time the Member's coverage under the Benefit Plan became effective. Please call the Plan's Member Service Department at (866) 859-7525 to see if you may be eligible for this benefit. You may request a copy of the Plan's Continuity of Care Policy from the Plan's Member Service Department. You must make a specific request to continue under the care of your non-Participating Provider. The Plan is not required to continue your care with your non-Participating Provider if you are not eligible under the Plan's Continuity of Care Policy or if the Plan cannot reach an agreement with your non-Participating Provider on the terms regarding your care in accordance with California law.

REFERRALS

The Plan provides referral to the following specialties for covered services: Periodontics, Endodontics, Oral Surgery, and Pedodontics.

The Member must first visit his or her Primary Care Dentist for evaluation of Member's case. If the Primary Care Dentist determines the Member requires a referral to a Specialist, the Member's Primary Care Dentist must initiate a referral to a Specialist on behalf of the Member. The Primary Care Dentist must submit the referral request to the Specialty Referral Department of the Plan, using the Specialist Referral Form. The process used by the Plan to review requests for Specialty Referrals and other benefits are available from the Member Service Department.

Please Note: If the request for Specialist services is not made in compliance with the foregoing (member goes to a specialty provider directly, without obtaining a referral from their primary care dentist), the Member will be responsible for the Specialist's full usual and customary fees for any such services rendered.

AUTHORIZATION OR DENIAL OF SERVICES

Members and Participating Providers are notified of authorizations and denials of Specialist Referral Requests as

follows: For routine referrals, the decision to approve, modify, or deny requests by a Member's Primary Care Dentist for specialty referrals will be made in a timely fashion. The Plan will take into consideration the Member's condition. Referrals will be processed within five (5) business days of the Plan's receipt of the information that is reasonably necessary and requested by the Plan to make the determination.

When the Member's condition is urgent, such that the Member faces an imminent and serious threat to his or her dental health, the Plan shall expedite the review process. The Plan shall make a decision within 72 hours of receipt of all necessary information.

If the Plan cannot approve or deny a request for authorization within the 72 hours or 5 business day timeframes the Plan shall immediately notify the referring Primary Care Dentist and the Member, in writing, that the Plan cannot make a decision within the required timeframe and specify the reason for the delay. The Plan shall also notify the Member of the anticipated date when a decision will be made. Once the Plan receives necessary information, the Plan shall approve or deny the request for authorization within the above timeframes for a routine or urgent decision, whichever applies.

The requesting Participating Provider will be notified by telephone or facsimile within 24 hours of the Plan's decision. The decision will be communicated to the requesting Participating Provider and the Member in writing within 2 business days.

If the referral is approved, the Plan shall specify in the notice the specific dental care service approved. Upon receipt of notification of approval, the Member may contact the Specialist to schedule an appointment. The Specialist will provide Specific Specialized Dental Care for the Copayment listed in the Schedule of Benefits. The Specialist will submit the claim for payment to the Plan and the Member shall be responsible for payment of the Copayment, as applicable.

If the referral is denied the Plan shall specify the reason(s) for the decision. The Plan shall include the criteria or guideline used, and the clinical reasons for the decision regarding medical necessity. The Plan shall include the name and direct telephone number or extension of the dental professional who made the decision at the Plan. Members are also notified in the denial notices as to how they may file a grievance with the Plan.

In cases where the request is retroactive (i.e., where the Member has already obtained the services from the Specialist), the Participating Provider and Member shall receive written notification of the Plan's decision within thirty (30) calendar days from the Plan's receipt of the information

that is reasonably necessary to make the determination.

In the case of concurrent review (i.e., where the care is already underway), care shall not be discontinued until the Member's treating Participating Provider has been notified of the Plan's decision and a care plan has been agreed upon by the treating Participating Provider that is appropriate for the dental needs of that Member.

FEES AND CHARGES

Premiums

The Plan shall provide or arrange for the provision of the Covered Services specified in the Benefit Plan. The Group shall pay the Prepayment Fee set out on the last page of the Schedule of Benefits.

The Prepayment Fee must be paid by Group at the Plan's address set out on the first page of this Evidence of Coverage Booklet. Member should consult Group for specific information regarding any sums to be withheld from the Member's salary or to be paid by Subscriber to the Group.

Copayments

In addition to the monthly Prepayment Fees, if any, you will pay a Copayment for those procedures or services listed in the Schedule of Benefits. All Copayments are paid by the Member directly to the Participating Provider. All Covered Services are listed in the Schedule of Benefits regardless of whether a Copayment applies. Those Covered Services that do not require a Copayment are designated in the Schedule of Benefits as "No Copayment."

Liability for Payment

Refer to ACCESS TO SERVICES Section - Liability of Member for Payment.

ELIGIBILITY AND ENROLLMENT

Eligibility

The determination of who is eligible to participate and who is actually participating in the Benefit Plan shall be decided by the Group and the Plan. Any disputes or inquiries regarding eligibility, including rights regarding renewal, reinstatement and the like, if any, should be directed to the Group.

Effective Date of Coverage

Coverage shall commence on the date specified in the Agreement for all Members enrolled as of the commencement date of the Agreement. Coverage shall commence for all new Members on the first day of the month following the Plan's receipt of eligibility for such new Members.

The following provisions apply to all Members who are enrolled in the Benefit Plan:

Dependents shall also include all children up to the age of 26 years, even if they are married, do not live with you, or are not students, whether they were previously enrolled on your plan or not. The spouse or children of your adult children are not eligible for dependent coverage.

Dependents shall also include all newborn infants whose coverage shall commence from the moment of birth and all natural, Domestic Partner's, adopted, foster, and stepchildren whose coverage shall commence from the date of legal custody or placement.

Coverage shall not terminate while a dependent child is and continues to be:

- (a) Incapable of self-sustaining employment by reason of a physically or mentally disabling injury, illness, or condition; or
- (b) Chiefly dependent upon the Subscriber for support and maintenance.

At least ninety (90) days prior to a child reaching the limiting age, the State will send notice to the Subscriber that coverage for the dependent child will terminate at the limiting age unless proof of incapacity and dependency is provided within sixty (60) days of receipt of notice. The State shall determine if the child meets the conditions above, prior to the child reaching the age limit. Otherwise, coverage of the child will continue until the State makes its determination. After two (2) years following the child reaching the limiting age, the Plan may request proof of continuing incapacity or dependency, but not more often than yearly.

If you are enrolling a disabled or dependent child for new coverage, the Plan may request initial proof of incapacity and dependency of the child, and then yearly, to ensure that the child continues to meet the conditions above. The Subscriber

must provide the Plan with the requested information within 60 days of receipt of the request. The child must have been covered as a dependent of the Subscriber or spouse under a previous health plan at the time the child reached the age limit.

No person shall be eligible as a Dependent who is eligible as a Subscriber, nor may any person be an eligible Dependent of more than one Subscriber.

Identification Card

The Plan issues each Member an identification card to be presented by the Member at the time that services are to be rendered by the Participating Provider. If an ID Card is lost, please call the Member Services Department at (866) 859-7525 to request a new one. If a Member contacts us to change his or her assigned Primary Care Dentist, a new ID card will be mailed to the Member with the new Primary Care Dentist's name, address and telephone number.

COVERED SERVICES

Benefits

The Plan provides coverage to Members as set forth in this Evidence of Coverage Booklet. Such coverage will be provided when necessary for the dental health of a Member in accordance with professionally recognized standards of dental practice, subject to the Exclusions, Limitations, and other terms and conditions set out in this Evidence of Coverage Booklet. The Schedule of Benefits establishes the Covered Services which are available for no Copayment (designated as "No Cost" in the schedule), and those services for which Members are obligated to pay a Copayment. The amount of the Copayment for specific Covered Services is set forth in the Schedule of Benefits. Members are financially responsible for all charges incurred from non-covered services.

The descriptive categories of Covered Services that correspond to the categories set forth in the Schedule of Benefits, together with references to Exclusions or Limitations specific to each category of services follows. To locate the specific Covered Services of this Benefit Plan for a category of services described in the Evidence of Coverage Booklet, refer to the corresponding category heading in the Schedule of Benefits.

Please Note: Refer to "EMERGENCY AND URGENT CARE" Section of this Evidence of Coverage Booklet for Emergency

Dental Care available under the Plan, and how to access those services. Refer to “REFERRALS” Section of this Evidence of Coverage Booklet for important information regarding the scope of Specialist services available under the Benefit Plan, and how to access those services.

A. Diagnostic – Clinical examinations, radiographs, and other diagnostic tools used in conjunction with the Member’s health history in order to evaluate necessary dental treatment. Refer to the “Diagnostic” category on your Schedule of Benefits to determine what specific procedures are Covered Services and their Copayment amounts.

Clinical examinations may include the following:

1. Comprehensive Oral Evaluation – A comprehensive evaluation of a Member’s dental health needs. This includes evaluating and recording a Member’s dental and medical history and a general health assessment, including such things as dental caries, missing or unerupted teeth, restorations, occlusal relationship, periodontal conditions (including periodontal charting), and hard and soft tissue anomalies.
2. Limited Oral Evaluation – An evaluation limited to a specific oral health problem or complaint. This may require interpretation of information acquired through additional diagnostic procedures. Typically, Members receiving this type of evaluation present with a specific problem and/or dental emergencies, trauma, acute infections, etc.
3. Detailed and Extensive Oral Evaluation – A detailed and extensive problem-focused evaluation based on the findings of a comprehensive oral evaluation, and development of a treatment plan for the specific problem evaluated. The condition requiring this type of evaluation should be described and documented.
4. Periodic Oral Evaluation – An evaluation performed to determine any changes in a Member’s dental and medical health status since a previous comprehensive or periodic evaluation. This includes periodontal screening and may require interpretation of information acquired through additional diagnostic procedures.

An initial visit shall include one of the following:

- Comprehensive Oral Evaluation
- Limited Oral Evaluation
- Detailed and Extensive Oral Evaluation

5. Radiographs/Diagnostic Imaging – Radiographs are primarily for clinical purposes; they represent an important diagnostic aide. A radiographic exam is a combination of periapical, bitewing, panoramic films or other views selected for a Member based on need. The number and type of radiographs in any examination will vary according to the needs of the Member.

6. Pulp Vitality Test – Assessment of vitality of the pulp tissue which occupies the pulp cavity of the tooth.

B. Preventive – Those procedures that aid in the prevention of dental and oral disease. Refer to the “Preventive” category of your Schedule of Benefits to determine which specific procedures are Covered Services and their Copayment amounts.

Preventive Services may include the following:

1. Prophylaxis (Adult and Child) – These cleanings include scaling and polishing of the crown portion of exposed teeth in the mouth and is the treatment for the removal of stain, plaque, and calculus (tartar) above the gum line. It is not intended to be treatment for active periodontal disease where subgingival (beneath the gum line) scaling and root planing is usually required.

2. Topical Fluoride Treatment – Application of topical fluoride to aid in the prevention of caries formation.

3. Nutritional Counseling for Control of Dental Disease – Counseling on food selection and dietary habits as a part of the treatment and control of periodontal disease and caries.

4. Oral Hygiene Instruction – Instructions for home care. Examples include tooth brushing technique, flossing, and use of special oral hygiene aids.

5. Sealants – The application of sealants to pit and fissure areas as a measure in the prevention of caries.

6. Space Maintainers – passive appliances designed to prevent tooth movement.

C. Restorative Services – Those procedures used to repair and restore the natural teeth to healthy condition. Refer to the “Restorative Services” category of your Schedule of Benefits to determine which specific procedures are Covered Services and their Copayment amounts.

1. Amalgam and Resin-Based Composite Restorations – Those procedures that include amalgam or resin-based composite restorative material used in order to repair and restore the natural teeth to healthy condition.

2. Crowns – Single Restoration Only – Those procedures that include gold, ceramic, porcelain and porcelain fused to metal in covering the tooth. If porcelain or composite materials are used in the fabrication of molar crowns, the member is responsible for an additional \$75 above the copayment. An additional charge of \$75 will also be applied for any procedure using noble or high noble metal.

3. Other Restorative Services –

a) Re-cementation of crowns – Use of adhesive material to reattach a crown that is dislodged.

b) Prefabricated Stainless Steel and Resin Crowns

c) Sedative filling – Temporary restoration intended to relieve pain.

d) Post and core buildup – Material placed within the root canal or in the tooth preparation for a crown when there is insufficient tooth strength and retention for the crown procedure.

D. Endodontics – Those procedures that involve treatment of the pulp, root canal and roots. Refer to the “Endodontics” category of your Schedule of Benefits to determine which specific procedures are Covered Services and their Copayment amounts.

1. Pulp Capping – Procedure in which exposed, or nearly exposed pulp is covered with a dressing that protects the pulp and promotes healing and repair.

2. Pulpotomy – Removal of a portion of the pulp to maintain the vitality of the remaining portion by means of a dressing.

3. Root Canal Therapy – The treatment of diseases and injuries of pulp and the root canal, and placement of the root canal filling.

4. Apicoectomy – A surgical procedure to repair the damages to the root surface.

E. Periodontics— Those procedures that involve the treatment of the gum and bone supporting the teeth and the management of gingivitis (gum inflammation) and periodontitis (gum disease). Refer to the “Periodontics” category of your Schedule of Benefits to determine which specific procedures are Covered Services and their Copayment amounts.

1. Periodontal Services (Surgical). The following Periodontal Services (Surgical) are Covered Services:

- a) Gingivectomy – Removal of part of the gingival margin resulting in exposure of more tooth structure.
- b) Osseous Surgery – Surgical procedure involving the reshaping of the bone to achieve a more healthy and physiologic status.

2. Periodontal Services (Non-surgical)

Scaling and Root Planing - Instrumentation of the crown and root surface of the teeth to remove plaque, calculus (tartar), and contaminated connective tissue from these surfaces.

F. Prosthodontics, Removable— Replacement of lost teeth by a removable prosthesis and the maintenance of those appliances. Refer to the “Prosthodontics (Removable)” category of your Schedule of Benefits to determine which specific procedures are Covered Services and their Copayment amounts.

1. Complete and Partial Dentures – Full or partial dentures are a Covered Service when dentures are necessary for the dental health of the Member consistent with professionally recognized standards of dental practice.

Replacement of an existing appliance will be covered if the appliance is over three years old. The three-year limitation does not apply to services rendered while the Member was not covered, to clinically defective dentistry, or when replacement is necessary for the dental health of the Member consistent with professionally recognized standards of dental practice.

2. Tooth Additions and Repair to Existing Dentures – When required because of loss of natural teeth, tooth addition to existing dentures is covered by the Plan. Replacement of missing or broken denture teeth, and repairs to the denture base are also covered.

3. Denture Reline and Rebase – The process of refitting a denture by resurfacing the tissue side of the denture, or by replacing the base material of the denture. Relining and

rebasement existing dentures are also Covered Services.

4. Interim Prosthesis – A provisional prosthesis designed for use over a limited period of time, after which it will be replaced by a more definitive restoration. If a Member receives an interim partial or interim complete denture while a permanent prosthetic appliance is being made, the Member will only be charged the Copayment for the permanent prosthetic appliance according to the accompanying Schedule of Benefits.

G. Prosthodontics, fixed (Fixed Partial Dentures or Bridges) – Replacement of lost teeth by fixed prosthesis is a Covered Service. Refer to the “Prosthodontics, Fixed” category of your Schedule of Benefits to determine which specific procedures are Covered Services and their Copayment amounts.

1. Fixed Partial Denture Pontics - (an artificial tooth on the Fixed Partial Denture) and Abutment Crowns (an artificial crown made to support a Fixed Partial Denture, and which is attached to the Fixed Partial Denture Pontic via a retainer) used in the fabrication process of Fixed Partial Dentures are Covered Services. If porcelain or composite materials are used in the fabrication of molar crowns, the member is responsible for an additional \$75 above the copayment. An additional charge of \$75 will also be applied for any procedure using noble or high noble metal.

2. Fixed Partial Denture Services –

- a) Recementation of Fixed Partial Dentures – Use of adhesive material to reattach a Bridge that is dislodged.
- b) Post and Core Buildup – Material placed within the root canal or in the tooth preparation for a crown when there is insufficient tooth strength and retention for the crown procedure.

H. Oral Surgery – Those procedures that involve the extraction of teeth and other surgical procedures as listed in the Schedule of Benefits. Oral Surgery procedures not specifically identified in the Schedule of Benefits are not covered. Refer to the “Oral and Maxillofacial Surgery” category of your Schedule of Benefits to determine which specific procedures are Covered Services and their Copayment amounts.

1. Extractions – Removal of teeth or parts of teeth.

2. Other Surgical Procedures.

I. Orthodontic Treatment – The Plan's orthodontic benefit covers only basic orthodontic treatment to resolve malocclusion and establish optimal dental and facial esthetics. Orthodontic treatment may involve the transitional or permanent dentition. All orthodontic services must be provided by a Participating Provider to be covered under the Benefit Plan. Refer to the "Orthodontics" category of your Schedule of Benefits to determine which specific procedures are Covered Services and their Copayment amounts.

1. Limited Orthodontic Treatment: Orthodontic treatment with a limited objective, not involving the entire dentition. It may be directed at the only existing problem in which a decision is made to defer or forego more comprehensive therapy. An example of this type of treatment would be treatment in one arch only to correct crowding or for closure of space(s). Limited Orthodontic treatment is a benefit for treatment of Transitional, Adolescent, and Adult Dentition. Limited Orthodontic Treatment is not a Covered Service unless specifically identified in the Schedule of Benefits.

2. Comprehensive Orthodontic Treatment: The goal of the comprehensive Orthodontic treatment is improvement of the alignment of the teeth, establishment of optimal interdigitation of the upper and lower teeth, and improvement of functional and esthetic relationships of teeth and jaw. Comprehensive Orthodontic treatment is a benefit for treatment of Transitional, Adolescent, and Adult Dentition.

3. Start-Up Fees: The copayment for orthodontic treatment does not include start-up fees. There is an additional copayment of \$250 for start-up fees. The start-up copayment includes the initial orthodontic consultation, and the retention (retainer) phase, including any adjustments necessary for the retainers.

Orthodontic Treatment After Termination

If a Member is receiving Orthodontic treatment at the time he or she is terminated from the Plan, the Member can continue receiving care from a Participating Provider for the following continuation fee:

- If up to 12 months of treatment has been completed at the time of termination: \$400.00;

- If between 12 and 18 months of treatment has been completed at the time of termination: \$300.00; or
- If 18 months or more of treatment has been completed at the time of termination: \$200.00.

The continuation fee is in addition to the original Orthodontic Treatment Copayment of your Benefit Plan as identified in the Schedule of Benefits. Members will be given the opportunity to pay the continuation fee over the remaining period of treatment. This continuation fee, as well as any outstanding balance at the time of termination, is payable to the Participating Provider. If the Member relocates to an area outside the geographic area served by the Plan, and is unable to receive treatment from a Participating Provider, continuing orthodontic coverage under the Plan ceases and the Member will have no further Orthodontic benefit from the Plan. (Copayments for retention and post-treatment records are still applicable.)

TERMINATION, RESCISSION AND CANCELLATION OF COVERAGE

Termination of Benefits

- A. Upon termination of a Subscriber's employment or membership with the Group, Member, as well as his/her Dependents, shall continue to be eligible to receive services until the last day of the month in which the Subscriber's termination occurred (See COBRA section for continuation).
- B. Continuing coverage under this Benefit Plan is subject to the terms and conditions of the Agreement.
- C. Pursuant to Section 1365(b) of the Knox-Keene Act, any Member who alleges his/her enrollment has been canceled or not renewed because of his/her health status or requirements for services may request review by the Director of the Department of Managed Health Care.
- D. The Plan may terminate Member's enrollment in this Benefit Plan if Member knowingly provides false information on his or her enrollment form, or fraudulently uses services or facilities of the Plan or providers, or knowingly allows another person to do so. Termination is effective 30 days after Notice of Cancellation, Rescission, or Nonrenewal is sent to Member. A Notice of End of Coverage shall be sent to Member no later than five (5) calendar days after the date of coverage ended. If coverage is terminated for any

of the above reasons, Member forfeits all rights to COBRA Continuation Coverage or to enroll in the Plan's Individual Conversion or other benefit plans in the future. The Plan does not provide for Member reinstatement following termination of individual membership.

Note: If the Agreement with your Group is terminated by the Plan, reinstatement of the Group's Agreement with the Plan is subject to all terms and conditions of that Agreement.

E. Participating Providers shall complete all procedures that were commenced prior to the date of the Member's termination, except for orthodontic treatment. Post termination arrangements for continuation of orthodontic treatment are described in "BENEFITS - Orthodontics section.

When the Agreement between Western Dental and Group is terminated, all Members covered under the Agreement become ineligible for coverage. The Agreement between Western Dental and Group may be terminated in any of the following circumstances:

- Fraud or intentional misrepresentation of material fact under the terms of this Agreement,
- The Member no longer resides, lives, or works in the Plan's service area,
- Violation of a material contract provision relating to employer contribution or group participation rates by the contract holder or employer,

If the Plan ceases to provide or arrange for the provision of health benefits for new health care service plan contracts, and
If the Plan withdraws from the market.

Enrollees subject to cancellations and nonrenewals for nonpayment of premiums will be sent a Notice of Start of Grace Period. These enrollees are entitled to a grace period of at least 30 consecutive days beginning the day the Notice of Start of Grace Period is dated, prior to cancellation, as follows:

The grace period may not begin sooner than the day after the last date of paid coverage.

A plan shall provide coverage pursuant to the terms of the contract during the entire grace period.

Upon determining that an enrollee, subscriber, or group contract holder has failed to make a premium payment by the due date, the plan shall send a Notice of Start of Grace Period to the enrollee, subscriber, or group contract holder, notifying the recipient that a payment delinquency has triggered a grace period starting from the day the Notice of Start of Grace Period is dated.

The plan shall continue the enrollee, subscriber, and/or group contract holder's coverage uninterrupted pursuant to the plan contract upon payment of all outstanding premium amounts at any time before the expiration of the grace period.

The enrollee, subscriber, or group contract holder is financially responsible for any and all premiums and any copayments, coinsurance, or deductible amounts obligated under the plan contract, including those incurred for services received during the grace period.

The plan shall send a Notice of Start of Grace Period to each subscriber in a group contract unless:

- The plan contract requires the group contract holder to promptly send any such notice to each subscriber; and
- The plan sends the notice to the group contract holder designated in the plan contract.

In the event the plan, after compliance with all timing and notice requirements of this section, fails to receive all outstanding premium amounts from the enrollee, subscriber, or group contract holder on or before the last day of the grace period, as specified in the Notice of Start of Grace Period, coverage may be cancelled prospectively only after the expiration of the entire grace period.

Termination of coverage is effective for all Members, including those who are hospitalized or undergoing treatment for an ongoing condition. According to the terms of the Agreement, the Group is responsible for notifying you if and when the Agreement is terminated for any reason, including the non-payment of Prepayment Fees, and for providing you with a copy of the Notice of End of Coverage provided to the Group by the Plan.

F. Upon termination of a Participating Provider's contract, the Plan shall be liable for Covered Services rendered by such Participating Provider, other than for Copayments and excluded services, to a Member who retains eligibility under the Agreement or by operation of law, and who was under the care of such Participating Provider at the time of such

termination, until the services being rendered to the Member by such Participating Provider are completed, unless the Plan makes reasonable and dentally appropriate provisions for the assumption of such services by a Participating Provider.

RENEWAL AND REINSTATEMENT OF COVERAGE

Renewal Provisions

The Plan has contracted to provide Covered Services for a period as specified in the Agreement. Thereafter, the Agreement may be renewed, with or without amendments, as specified in the Agreement. The Group may terminate the Agreement by giving the other party sixty (60) days written notice prior to the termination date of the Agreement. Failure to give such notice shall automatically renew the Agreement for a subsequent renewal term as specified in the Agreement. During the term of the contract, Plan may not increase the Prepayment Fee, the Copayment amounts paid by the Members, or decrease the Covered Services in any manner during a contract term without a prior written agreement between Group and Plan.

CONTINUATION OF BENEFITS

Individual Continuation of Benefits

1. Loss of Group Eligibility-The Member who becomes ineligible for group coverage may apply within 30 days of notice of ineligibility to continue Benefit Plan coverage. The terms and conditions under the Agreement in which such Member was enrolled shall continue in effect with the following exceptions: Notices and distribution of materials as required will be delivered directly to the Member; Member shall pay the applicable monthly premium in effect at the time the application to continue coverage is approved by the Plan. Such extension of coverage shall apply to the Dependent(s) of the converting Members upon the same terms and conditions as applied to the converting Member. Such application may be accepted or rejected at the option of the Plan; no automatic right of individual continuation of benefits exists.
2. Loss of Eligibility Due to Termination of Subscriber Agreement - Plan reserves the right to offer conversion privileges to the Subscriber who becomes ineligible due to the termination of the Agreement. Should such conversion be offered to the Subscriber, application must be made

within 30 days of notice of ineligibility to continue Benefit Plan coverage. The terms and conditions under the Agreement in which such Subscriber was enrolled shall continue in effect with the following exceptions: Notices and distribution of materials as required will be delivered directly to the Subscriber; Subscriber shall pay the applicable monthly premium in effect at the time the application to continue coverage is approved by the Plan. Such extension of coverage shall apply to the Dependent(s) of the converting Subscriber upon the same terms and conditions as applied to the converting Subscriber.

COBRA

Federal COBRA Information

Pursuant to COBRA legislation, this information will serve to advise you of certain rights which you or your family members may have to continuation of coverage under the Benefit Plan in the event of a termination of eligibility due to one of the following qualifying events:

1. Death of covered employee;
2. Termination of covered employee (other than for gross misconduct) or reduction in covered employee's hours of employment;
3. Divorce or legal separation of the covered employee from the employee's spouse;
4. Entitlement to Medicare benefits by the covered employee;
5. A Dependent child ceasing to be eligible for coverage as a Dependent child under the Benefit Plan.

For widows, divorced spouses, spouses of Medicare eligible employees, and Dependent children who become ineligible under the Benefit Plan, continuation coverage may be available for up to 36 months. Continuation coverage for terminated or reduced hour employees, and their eligible Dependents, may be available for up to 18 months.

A monthly premium must be paid by you to the Plan for the continuation coverage. The premium will be determined at the time of eligibility and will be subject to change; however, the premium charged to you will not exceed 102% of the premium

charged for active employees and/or Dependents in a comparable status. The continuation coverage will be the same as the coverage available to continuing employees,

regardless of your health at the time. Coverage under COBRA must begin on the date of the qualifying event.

Continuation coverage will not be available to you after:

1. You fail to make timely premium payments; or
2. You or your spouse or Dependent is covered under any other group health plan as the result of employment, re-employment, or remarriage; or
3. You or your spouse or Dependent becomes entitled to Medicare benefits; or
4. Your employer or former employer ceases to maintain the Benefit Plan for employees.

At the time of eligibility for continuation coverage, an election form will be provided to you by the Group. The form must be completed and returned by the date noted. You or your eligible family member must notify the Group of a divorce, legal separation, or loss of eligibility of a dependent child upon the occurrence of such event.

If you should have any questions about this benefit, please direct them to the Group.

You also have the option of obtaining coverage under an individual plan.

GRIEVANCE PROCEDURES

Complaints and Disputes

Any dispute, complaint, or request for information should be directed to the Plan at the following address by certified mail, return receipt requested:

WESTERN DENTAL SERVICES, INC.

P.O. Box 14227 Orange, CA 92863

Telephone calls should be made to the Plan at the following
number:(866) 859-7525

Grievance Procedures

Members are encouraged to contact the Plan at the telephone number listed above regarding any concerns they may have while obtaining services. The Plan maintains a grievance process to address these concerns. Member complaints or grievances can be made in person, at any Participating Provider's office, by obtaining a grievance form from and submitting it to the Plan, or by submitting the grievance using the Plan's website at www.westerndental.com. There is a representative at the Participating Provider's office or at the Plan's corporate office to aid the Member in filling out the grievance form. Completed grievance forms must be mailed to the Plan at the address listed above. Members will receive a written response within 30 days as to the disposition of the grievance.

The Department of Managed Health Care is responsible for regulating health care service plans. If you have a grievance against Western Dental, you should first telephone Western Dental at **1-866-859-7525**, and use Western Dental's grievance process before contacting the department. Utilizing this grievance procedure does not prohibit any potential legal rights or remedies that may be available to you. If you need help with a grievance involving an emergency, a grievance that has not been satisfactorily resolved by Western Dental, or a grievance that has remained unresolved for more than 30 days, you may call the department for assistance. You may also be eligible for an Independent Medical Review (IMR). If you are eligible for IMR, the IMR process will provide an impartial review of medical decisions made by a health plan related to the medical necessity of a proposed service or treatment, coverage decisions for treatment that are experimental, or investigational in nature and payment disputes for emergency or urgent medical services. The department also has a toll-free telephone number (**1-888-466-2219**) and a TDD line (**1-877-688-9891**) for the hearing and speech impaired. The department's Internet Website <http://www.dmhc.ca.gov> has complaint forms, IMR application forms, and instructions online. A Member may submit a complaint or grievance to the Department for review after the Member has participated in the Plan's grievance process for at least 30 days.

If the Member's grievance involves an imminent and serious threat to his or her health—including but not limited to, severe pain, potential loss of life, limb, or major bodily functions—the Member may submit the grievance to the Department without waiting 30 days. In such a situation, the Plan will immediately

inform the Member of his or her right to notify the Department of the complaint. In such a situation, the Plan also will provide the Member and, as appropriate, the Department with a written statement of the status or disposition of the complaint within three days of receipt of the grievance.

The Plan will provide written acknowledgement of receipt of a grievance within five (5) calendar days of receipt of the grievance. The acknowledgement will indicate that the grievance has been received, will include the date of receipt of the grievance, and will indicate the name of the plan representative and telephone number and address of the plan representative who may be contacted about the grievance.

Independent Medical Review (IMR)

You may also be eligible for an Independent Medical Review (IMR). If you are eligible for IMR, the IMR process will provide an impartial review of medical decisions made by a health plan related to the medical necessity of a proposed service or treatment, coverage decisions for treatment that are experimental, or investigational in nature and payment disputes for emergency or urgent medical services. The department also has a toll-free telephone number (**1-888-466-2219**) and a TDD line (**1-877-688-9891**) for the hearing and speech impaired. The department's Internet Website <http://www.dmhca.gov> has complaint forms, IMR application forms, and instructions online.

RIGHT TO SUBMIT GRIEVANCE REGARDING CANCELLATION, RESCISSION, OR NONRENEWAL OF YOUR PLAN ENROLLMENT, SUBSCRIPTION, OR CONTRACT.

If you believe your health care coverage has been, or will be, improperly cancelled, rescinded, or not renewed, you have the right to file a grievance with the plan and/or the Department of Managed Health Care.

OPTION (1) YOU MAY SUBMIT A GRIEVANCE TO YOUR PLAN.

You may submit a grievance to Plan by calling 1-866-859-7525 or online at www.westerndental.com or by mailing your written grievance to:

WESTERN DENTAL SERVICES, INC.
P.O. Box 14227
Orange, CA 92863

You may want to submit your grievance to the Plan first if you

believe your cancellation, rescission, or nonrenewal is the result of a mistake. Grievances should be submitted as soon as possible.

Plan will resolve your grievance or provide a pending status within three (3) calendar days. If you do not receive a response from the Plan within three (3) calendar days, or if you are not satisfied in any way with the Plan's response, you may submit a grievance to the Department of Managed Health Care as detailed under Option 2 below.

OPTION (2) YOU MAY SUBMIT A GRIEVANCE DIRECTLY TO THE DEPARTMENT OF MANAGED HEALTH CARE.

You may submit a grievance to the Department of Managed Health Care without first submitting it to the Plan or after you have received the Plan's decision on your grievance.

You may submit a grievance to the Department of Managed Health Care online at:

WWW.HEALTHHELP.CA.GOV

You may submit a grievance to the Department of Managed Health Care by mailing your written grievance to:

HELP CENTER
DEPARTMENT OF MANAGED HEALTH CARE
980 NINTH STREET, SUITE 500
SACRAMENTO, CALIFORNIA 95814-2725

You may contact the Department of Managed Health Care for more information on filing a grievance at:

PHONE: 1-888-466-2219

TDD: 1-877-688-9891

FAX: 1-916-255-524

ARBITRATION

Any and all disputes of any kind whatsoever, including, but not limited to, claims for dental malpractice (that is as to whether any dental services rendered under the Benefit Plan were unnecessary or unauthorized or were improperly, negligently, or incompetently rendered) between Member (including any heirs, successors, or assigns of Member) and Plan, except for claims subject to ERISA, shall be submitted to binding arbitration. Any such dispute will not be resolved by a lawsuit or resort to court process, except as California law provides for judicial review of arbitration proceedings. Member and Plan are giving up their constitutional rights to have any such dispute decided in a court of law before a jury and are instead accepting the use of binding arbitration by a single arbitrator in accordance with the rules of the American Arbitration

Association (the “AAA”). Arbitration, however, shall not be conducted by the AAA, and shall be conducted by another mutually agreed upon arbitration administrator. The parties will endeavor to mutually agree to the appointment of the arbitrator, but if such agreement cannot be reached within thirty (30) days following the date demand for arbitration is made, the arbitrator appointment procedures in the AAA Rules will be utilized.

Arbitration hearings shall be held in the California county in which the Member resides at the time of their initial enrollment, or at such other location as the parties may agree in writing. The arbitrator selected shall have the power to control the timing, scope and manner of the taking of discovery and shall further have the same powers to enforce the parties’ respective duties concerning discovery as would a Superior Court of California including, but not limited to, the imposition of sanctions. The arbitrator shall have the power to grant all remedies provided by California law. The parties shall divide equally the expenses of the arbitration administrator and the arbitrator. In cases of extreme hardship, the Plan may assume all or part of the Member’s share of the fees and expenses of the arbitration administrator and the arbitrator, provided the Member submits a hardship application to the arbitration administrator. The approval or denial of the hardship application will be determined solely by the arbitration administrator.

The arbitrator shall prepare in writing an award that includes the legal and factual reasons for the decision. The arbitration decision is final and binding on the parties, and the award may only be vacated or corrected pursuant to California law. The Federal Arbitration Act, 9 U.S.C. §§ 1-16, shall also apply to the arbitration.

MISCELLANEOUS

Coordination of Benefits

Coordination of Benefits is a process, regulated by law, which determines the financial responsibility for payment when a person has group health care coverage under more than one plan. The following rules are used to determine which plan is primary and which is secondary for payment. The rules define the “Coordination of Benefits.”

- A. Member may be covered as an employee by his/her employer and as a dependent by his/her spouse’s

employer. The plan that covers the Member as an employee (the policyholder) has primary plan benefits.

- B. If a child is covered as a dependent under both parents' coverage (and parents are not separated or divorced), the plan of the parent with the earliest birthday in the year has primary responsibility for plan benefits.
- C. If a child of divorced or separated parents is covered as a dependent under the parents' coverage, benefits are determined in this order:
 - 1. The parent's plan that has custody of the child.
 - 2. The spouse's plan of the parent who has custody of the child.
 - 3. The parent's plan not having custody of the child.
- D. The benefits of a program which covers a person as an active employee are determined before those of a program which covers a person as a laid-off or retired employee.
- E. If spouses/dependents are covered by the Benefit Plan in another managed care program, the Participating Provider must accept the coverage that best benefits the Member.
- F. If none of the above rules determine the order of benefits, the plan which has covered the employee the longest has primary plan benefits.
- G. If a Member has a conversion plan with the Plan, and then obtains dental coverage through a new employer, the group plan is billed as if there were no other coverage. The conversion plan is not subject to Coordination of Benefits.

WHEN THE PLAN IS PRIMARY, Your Participating Provider can:

- Submit to the insurance company on a secondary basis at Participating Provider's usual and customary rates, but indicating the out-of-pocket Benefit Plan Copayment for procedures performed.
- Accept payment from the secondary insurance company equal to the Member Benefit Plan Copayments.
- Only bill the Member if the insurance pays an amount less than the Benefit Plan Copayment. The Participating

Provider may bill Member for the Copayment.

WHEN THE PLAN IS SECONDARY, Your Participating Provider can:

- Bill primary coverage for all procedures at Participating Provider's usual and customary rates.
- When the Benefit Plan is secondary, the Participating Provider is entitled to keep all proceeds from the primary plan but must waive the Benefit Plan Copayment if the reimbursement exceeds the Copayment responsibility. However, if the other plan benefit is less than the Copayment, the Participating Provider or the office may collect the difference from the Member.

Participation in Public Policy

The Plan welcomes Member participation on its Public Policy Committee, which meets quarterly at the Plan's corporate offices in Orange, California. In order to be considered for membership, please write or call the Plan's Member Service Department at 866-859-7525.

Filing Claims

In the event that Member requires Emergency Dental Care, Member should contact his or her Participating Provider to schedule an immediate appointment. For urgent dental conditions that occur after hours or on weekends, Member must contact the Participating Provider for instructions on how to proceed. If after contacting the Participating Provider the Member is advised that the Participating Provider is not available, Member may obtain Emergency Dental Care from any Participating Provider within a 50 miles area or a licensed dentist in the area where such dental emergency occurs if greater than 50 miles. Members may contact the Plan for assistance to obtain an emergency appointment from a Participating Provider. Treatment by Participating Providers will be provided at the applicable Copayment listed in the Schedule of Benefits. However, there is a one hundred dollar (\$100) maximum allowable benefit for Emergency Dental Care provided by a non-Participating Provider. The Plan requires an itemized statement of services from the non-Participating Provider or the Member within one-hundred eighty (180) days from the date of service for verification of benefit reimbursement.

The Member must include the itemized statement of services, the Member's name, address, Member ID number, dates of service, treating provider's name, address, and telephone number, and a statement of the problem, and mail it to:

Western Dental Services, Inc.
Attn: Specialty Referrals/Claims Department
P.O. Box 14227
Orange, California 92863

The Member should retain a copy of the information, and the Plan will either send the Member a check or explain any denial within thirty (30) business days of the Plan's receipt of the Member's claim.

Confidentiality of Medical (Dental) Records

A STATEMENT DESCRIBING THE PLAN'S POLICIES AND PROCEDURES FOR PRESERVING THE CONFIDENTIALITY OF MEDICAL RECORDS IS AVAILABLE AND WILL BE FURNISHED TO YOU UPON REQUEST

Requests for Confidential Communications

The Confidentiality of Medical Information Act was recently amended to provide you with greater confidentiality when you receive sensitive health care services.

If you live in California, you have the right to request all communications regarding your receipt of sensitive services be sent directly to you (either to your contact information on file or to a designated alternative address, email, or phone number).

Examples of confidential communications include:

- Bills and attempts to collect payment;
- Notices of adverse benefits determinations;
- Explanation of benefits notices;
- A plan's request for additional information regarding a claim;
- Notices of a contested claim;
- The name and address of a provider, description of services provided, and other information related to a visit; and
- Any written, oral, or electronic communication from a plan that contains protected health information.

Requests for confidential communications can be submitted to:

By regular mail: Western Dental Member Services
530 S Main Street
Orange, CA 92868

By electronic mail: privacyofficer@westerndental.com

By phone: 1-866-859-7525

We will process requests received by email or phone within seven (7) calendar days, and requests received by first-class mail within fourteen (14) calendar days. If you contact us, we will confirm receipt of your request and give you an update on its status.

You do not need to obtain the permission of the primary subscriber or other enrollee in order to receive sensitive services or submit a claim for sensitive services if the protected individual has the right to consent to care.

We will permit and accommodate requests for confidential communications in the form and format requested, if readily producible in the requested form and format, or at alternative locations.

We will not disclose information related to sensitive health care services you receive to the primary subscriber or any plan enrollees without your express authorization.

Your enrollment or coverage will not be affected by exercising this right.

Requests will be valid until you revoke the request or submit a new one. Requests for confidential communications will apply to all communications that disclose medical information or provider name and address related to your receipt of medical services.

A STATEMENT DESCRIBING WESTERN DENTAL SERVICES, INC. POLICIES AND PROCEDURES FOR PRESERVING THE CONFIDENTIALITY OF MEDICAL RECORDS IS AVAILABLE AND WILL BE FURNISHED TO YOU UPON REQUEST.

i "Sensitive Services" means all health care services related to mental or behavioral health, sexual and reproductive health, sexually transmitted infections, substance use disorder, gender affirming care, and intimate partner violence, and includes services described in Sections 6924, 6925, 6926, 6927, 6928, 6929, and 6930 of the Family Code, and Sections 121020 and 124260 of the Health and Safety Code, obtained by a patient at or above the minimum age specified for consenting to the service specified in the section.

Nondiscrimination Notice

Western Dental Services, Inc. ("Western Dental") complies with applicable State and Federal civil rights laws and does not

discriminate on the basis of age, race, creed, color, national origin, ancestry, disability, religion, sex, gender, gender identity or expression, sexual orientation, marital status, honorably discharged veteran or military status, or any other protected category.

Western Dental:

Provides appropriate aids and services, free of charge, to people with disabilities to communicate effectively with us, such as:

- Qualified sign language interpreters
- Qualified readers
- Assistance reading and completing patient paperwork
- Written information in other formats (e.g., which may include large print, audio, accessible electronic formats, and/or other formats)

Provides Assistance for Patients Who Have Mobility Disabilities:

While Western Dental generally expects patients to navigate its facilities and equipment without the need of physical assistance, Western Dental office staff will provide assistance to people with mobility disabilities upon request by the patient and to the extent the same may be accomplished safely for both patients and Western Dental staff.

Provides language assistance services, free of charge, to people whose primary language is not English, such as:

- Qualified interpreters
- Information written in other languages

If you need these services, contact the Western Dental's Member Service Department at 1-866-859-7525.

If you believe that Western Dental has failed to provide these services or discriminated in another way on the basis of race, color, national origin, age, disability, or sex, you can file a grievance/complaint in the following ways:

- By completing the grievance form directly on our website at www.westerndental.com or emailing a completed form to MemberServices@westerndental.com.
- By mailing or bringing in-person your written grievance form to the attention of WDS Member Service Department

at 530 S. Main Street, Suite 600, Orange, CA 92868.

- By calling our WDS Member Service Department toll-free at 1-866-859-7525 (TTY: 714-571-3624).

Please call the WDS Member Service Department at 1-866-859-7525 if you need help submitting or writing your grievance form.

You may also file a civil rights complaint with the U.S. Department of Health and Human Services, Office for Civil Rights. Complaint forms are available at <http://www.hhs.gov/ocr/office/file/index.html>.

You can mail it to:

U.S. Department of Health and Human Services
200 Independence Avenue, SW
Room 509F, HHH Building
Washington, DC 20201
Toll-free: 1-800-368-1019
TDD: 1-800-537-7697

You can also send it electronically to a website through the Office for Civil Rights Complaint Portal at: <https://ocrportal.hhs.gov/ocr/portal/lobby.jsf>.

Organ Donations

Donating organs and tissue provides many societal benefits. Organ and tissue donation allows recipients of transplants to go on to lead fuller and more meaningful lives. Currently, the need for organ transplants far exceeds availability. If you are interested in organ donation, please speak with your physician. Organ donation begins at the hospital when a Member is pronounced brain dead and identified as a potential organ donor. An organ procurement group will become involved to coordinate the activities.

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